

Kakis.

Trusted respite when a need arises.

A respite-activation platform for caregivers of Singapore's relatively healthy elderly — planned or urgent — layered on Vanguard's Pasir Ris ICCP pilot.



Mdm Halimah

Chaperone & companionship · Pasir Ris

Papa knows her — 6 visits together since May. No re-introduction needed.

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SGLN TECH · Demo Day · September 2026

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The question kept moving. We kept the receipts.

19 May



Companionship

"Help elderly in Singapore experience genuine daily connection."

Five family stories, one shared word: connection.

15 Jun



The reframe signal

NCSS's year-old HMW was sharper than ours. Aditi conceded it on the call.

We sat with the tension for ten days instead of pivoting on the spot.

25 Jun



Caregiver respite

"Offer on-demand respite to caregivers of the relatively healthy elderly."

Our field data + NCSS IDIs both pointed away from companionship-as-product.

8–11 Jul



Need-arises activation

"Activate trusted respite when a need arises — planned or urgent — on Vanguard's Pasir Ris ICCP pilot."

Vanguard named six crisis triggers on the 8th. On the 11th we caught crisis-only as too tight: planned respite is the same product, longer lead time.

Care plans work — until the day they break.

TUESDAY, 7:43 AM

Priya's helper has left. No notice.

Her father, 78, walks with a stick and takes diabetes medication at 2pm.

She has a 9am meeting and a son to fetch at 5.

Her options today: agencies quoting 2–3 weeks, a nursing home that can't take short stays, or annual leave — again.

She is the buyer, the user, and the one who breaks.

The urgent six — named by Vanguard, in the room:



Helper leaves suddenly

1–2 weeks of bridging care



Spouse hospitalised

the other senior is stranded



Discharge, no plan

hospital can't hold; family scrambles



Sudden decline

a fall, surgery — weeks of extra help



Loss of a spouse

grief + a care gap at once



Caregiver's own crisis

the carer needs cover too

The demand is real — and mostly hiding.

1M

seniors aged 60+ in Singapore

~30% non-robust (~300K)

15–30K

est. respite bookings / month nationwide

Vanguard's whiteboard extrapolation from Pasir Ris attach rates

2–3 wks

lead time at incumbent providers

confirmed by NCSS calling Homage & Jagami themselves

>50%

of likely beneficiaries don't know services exist

demand is latent, not observable — sizing from current uptake is a floor

3–4 in 20

requests Vanguard already turns away

supply-constrained — observed demand understates the ceiling

<24 hrs

what a crisis actually allows

no current option fits inside it

What the field said — before we believed it.

“Cannot be AI.”

Founder of a senior-tech company — who sells AI companionship for a living. Engagement decays once seniors detect the pattern.

So Kakis routes tech toward human connection, never in place of it. The AI works the back office.

“Consistency is the most important thing in whatever solutions.”

Caregiver of a grandmother with dementia. NCSS's IDIs agree: seniors repel new faces; adjusting takes months.

So matching is sticky by design — same kaki, same senior, and a shadow visit before any first solo appointment.

“Loneliness is when I wake up without any activities planned for the day.”

A senior who had joined everything — and still hit empty days.

So respite visits are structured around the senior's day: market runs, meals, mahjong — not generic ‘care hours’.

Three constraints shape everything we build.

1

Consistency is a hard constraint

Seniors actively repel unfamiliar helpers; adjustment takes months, not days. Rotation-friendly marketplaces fail at the first visit.

Design answer: sticky senior-kaki matches, and every first visit paired with Vanguard care staff — their proven pattern, made scalable.

2

The moat is not code

Luce already has AIC-accredited caregivers and same-day booking. A matching algorithm is not defensible — NCSS, Grab and Gojek all concluded the same.

Design answer: differentiate on what no commercial player has — ICCP integration, the multi-payer stack, and crisis activation as the product.

3

Willingness-to-pay is broken

Vanguard's session costs ~\$26 to deliver; it charges \$25–35 and many families still can't pay. Private pay alone cannot fund consistency.

Design answer: a multi-payer stack — private pay + healthcare fund + philanthropic capital + corporate partners — on one supply layer.

Kakis is not another marketplace.

It's an activation layer on infrastructure that already exists — Vanguard's Pasir Ris ICCP pilot, its trained micro-jobber pool, and its training pipeline at St. Luke's.



Activation layer

Planned or urgent, one front door. Six urgent triggers jump the queue — matched in hours, not 2–3 weeks. Planned bookings keep trusted supply busy between crises.



Care-planning wrapper

Every senior has a living care plan — meds, mobility, language, routines — so any kaki arrives briefed, and families stop re-explaining.

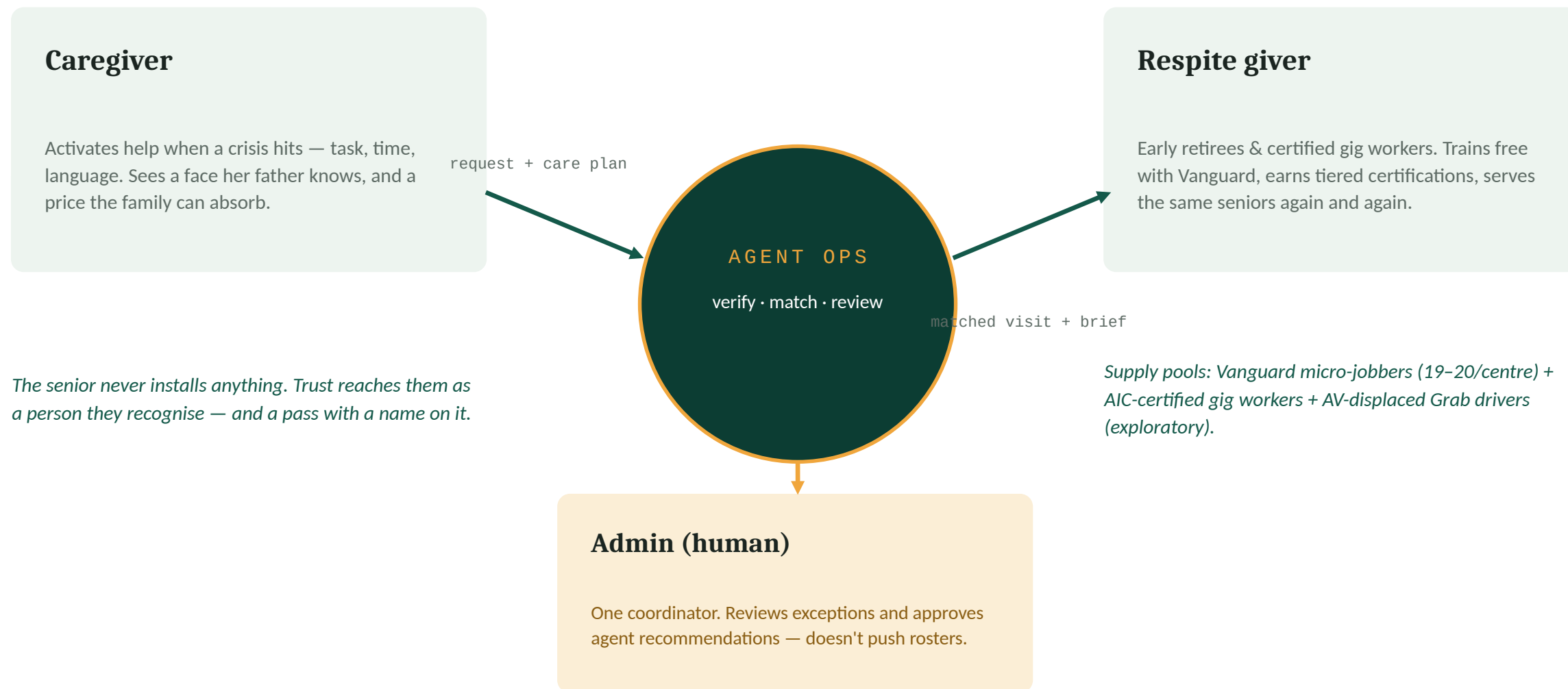


Certified-supply orchestrator

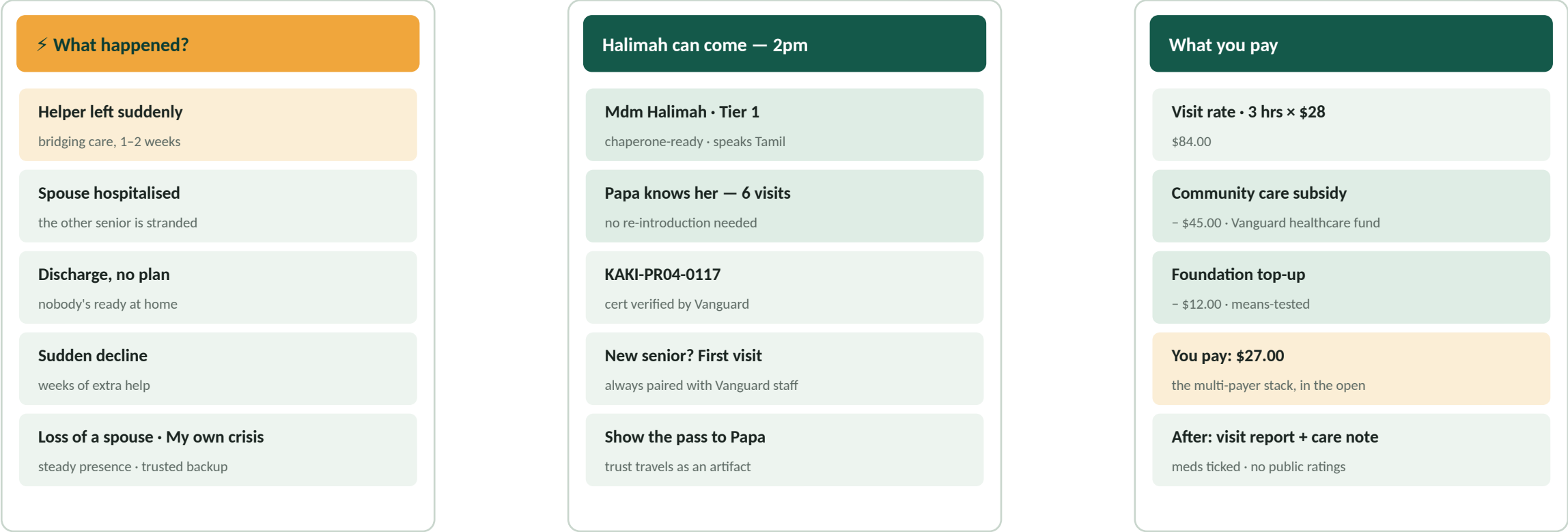
Vanguard-run training, St. Luke's certification, tiered skills. Task-typed matching: chaperone ≠ medicine administration.

Anchored: Angeline (Vanguard) volunteered a Pasir Ris pilot by year-end 2026, layered on the existing ICCP platform.

One loop, three sides — agents in the middle.



Priya: from crisis to a known face, in hours.



Urgent or planned: crisis jumps the queue, Book-ahead takes the everyday.

Regulars first. The pass makes trust visible.

Four payers, one receipt. Nothing hidden.

Mdm Tan: trained by Vanguard, certified at St. Luke's.

What can you help with?

Chaperone ✓
polyclinic, market, meals out

Companionship ✓
presence, games, walks

Wellness checks
meals, meds, safety drop-ins

Medicine admin — locked
locked until Tier 2 module

Dementia care — locked
locked until Tier 2 module

Skills gate tasks. Locked tiers are visible — the path up is the point.

Training path · Tier 1

CPR + AED — passed
external cert · St. Luke's Hospital

Mobility assistance — passed
half-day · Vanguard in-house

Working with seniors — passed
dialects, introductions, SOPs

Shadow visit — Thu 16 Jul
first real visit, with Vanguard staff

All modules free of charge
funded via NCSS / Vanguard pipeline

Vanguard's real modules. The shadow visit is the pre-relationship layer.

Visits near you

✂ Crisis · today 2–5pm · \$36
Mr Nathan — you've visited him 6×

Polyclinic escort · Tue · \$24
first visit — paired with staff

Mahjong kaki · Wed · \$22
Mr Silva, 76 · English

Only visits that match your certs
and your languages

My impact: 4 seniors · 27 hrs
consistency streak: 6 repeat visits

Regulars ranked first. Contribution framed above income.

41 bookings. 3 decisions. One coordinator.



Onboarding agent

Cross-checks Singpass, the St. Luke's cert registry and Vanguard's LMS. Shows its evidence trail.

This week: 12 verified · 2 flagged (one reused reference document caught)



Matching agent

Optimises consistency first (45%), then language, proximity, speed. Proposes weight changes with simulated impact.

This week: 34 auto-matched, regulars preserved · 1 exception — no Tamil speaker free Tue am



Quality agent

Reads every visit report and private care note. Surfaces patterns families miss.

This week: 18 reports read · 1 flag — a senior's fatigue across 3 visits → nurse check-in scheduled

Hard rules no agent can override

First visit to a new senior → paired with Vanguard staff · No public ratings or rankings of care staff (MOH/AIC policy) · Certification tier must cover the task — no exceptions · Urgent triggers jump the queue

Private pay can't fund consistency. Stack the payers.

ONE VISIT · 3 HRS · CHAPERONE

Visit rate (3 × \$28) \$84.00

Healthcare-fund subsidy – \$45.00

Philanthropic top-up (means-tested) – \$12.00

Caregiver pays **\$27.00**

4th payer: corporate partners — hotels donate low-season rooms for respite stays; employers add eldercare benefits.

The unit reality (Vanguard, today):

\$10–12/hr paid to micro-jobbers (gig market: \$20–22)

~\$26 fully-loaded cost per session

\$25–35/hr charged — and many families still can't pay

Kakis' pitch to the sector: today each payer is siloed by operator. The platform lets all four fund the same certified supply layer — that's the business case NCSS asked us to build.

The market, honestly: a floor, a signal, a ceiling.

FLOOR — observed

20–30

chaperone bookings/month across Vanguard's 4 centres — while turning away 3–4 in 20 requests, with zero marketing

SIGNAL — extrapolated

15–30K

bookings/month nationwide if Pasir Ris attach rates (5–10%) hold across 1M seniors — Vanguard's own whiteboard math

CEILING — modelled

\$425M

Grab's SAM for senior-adjacent services — a boundary on the space, not a forecast for respite

Why the floor understates: >50% of likely beneficiaries don't know services exist (NCSS), supply constraints suppress observed demand, and crisis demand never shows up in scheduled-service data. The pilot exists to replace this slide with measured numbers.

Pasir Ris, by year-end. Scoped, not hoped.

Vanguard brings

- The Pasir Ris ICCP sub-region (~15,000 seniors)
- 76-80 trained micro-jobbers across 4 centres
- Training pipeline + St. Luke's certification
- First-session care staff & phone-triage fallback

We bring

- The Kakis platform: activation, matching, agent ops
- Pilot design, KPI framework, measurement plan
- The multi-payer business case (NCSS's ask)
- Care-planning wrapper — the sector's named gap

We measure

- Urgent fill < 24 hrs; planned fill ≥ 95% (vs 3-day notice)
- First-visit success rate & senior acceptance
- Consistency: % repeat senior-kaki pairs ≥ 90%
- Ops cost per booking as volume scales past 30/mo

20 Jul

SGLN masterclass — concept walk-through

20 Aug

Prototype milestone — this build, pressure-tested

Sep

Demo Day — pilot design + business case

Q4 2026

Pasir Ris pilot live on ICCP

What could kill this — and what we're doing about it.

Nobody owns it in October

NCSS won't own it. Grab and Gojek declined. A demo without an owner is just a demo.

Vanguard's volunteered Pasir Ris pilot makes year one their operational scope.
Counterparty (HCLI Tech vs our own NGO) is our top open item — asked, not avoided.

Demand is latent, maybe imaginary

Ministry data doesn't show it; if it did, MOH would already fund it (Angeline's own words).

That's the point of the pilot: generate the demand data the ministry says it lacks. KPIs are designed as evidence for a funding case, not vanity metrics.

This space has a graveyard

NCSS's deck is "exactly the same as one year ago." Something keeps stalling everyone.

Diagnosis: ownership + a business case that doesn't close on private pay. Those are precisely the two things we're building first — not the app.

Two demand curves in one product

Planned respite (weeks of lead time, price-sensitive) and urgent respite (hours, price-tolerant) may want different UX, SLAs and prices — and planned is the incumbents' strongest ground.

One supply layer, one certification base, sticky matching binding both. The pilot tests both vectors; if the UX diverges, v2 splits the front door — not the platform.

We started with companionship. The evidence walked us here.

1

AIC demand data

The meeting that turns our extrapolation into evidence: MET waitlists, referral funnels, sub-region profiles.

2

A counterparty for the MOU

HCLI Tech as pilot counterparty — or a ruling that lets us stand up an NGO in time for a Q4 start.

3

Pasir Ris pilot scope by August

Service definitions, budget envelope and KPIs agreed with Vanguard while the offer is warm.

And the long game: every visit generates structured care data. Vanguard-certified humans today; the training substrate for assistive robotics in 3–5 years. The marketplace is the wedge — the data layer is the compounding asset.